



eCBHFA

Communicable disease prevention module

Volunteer guide for empowering communities



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Volunteer guide

The International Federation of Red Cross and Red Crescent Societies (IFRC) is the world's largest volunteer-based humanitarian network, reaching 150 million people each year through our 191 member National Societies. Together, we act before, during and after disasters and health emergencies to meet the needs and improve the lives of vulnerable people. We do so with impartiality as to nationality, race, gender, religious beliefs, class and political opinions.

Our strength lies in our volunteer network, our community-based expertise and our independence and neutrality. We work to improve humanitarian standards, as partners in development and in response to disasters. We persuade decision-makers to act at all times in the interests of vulnerable people. The result: we enable healthy and safe communities, reduce

vulnerabilities, strengthen resilience and foster a culture of peace around the world.

The IFRC is proud to work with USAID to help National Societies better prepare communities, National Societies, governments, civil society and the private sector to prevent, detect and respond to disease threats and play a significant role in preparing for future risks.

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Foreword

You are part of a global movement to protect every person from preventable illness. More and more, we are understanding that communicable diseases and disease outbreaks, begin and end in communities. When people get sick, before they go to a clinic, get laboratory tests, they are with their family and friends. As a disease spreads through a community, it is family members, friends and neighbours who are both most at risk of infection and have the most power to prevent the spread of a disease.

Red Cross and Red Crescent volunteers are embedded in communities, they are trusted family members, friends and neighbours. Because you, as a community volunteer, have a special role in your community, people may turn to you for advice when they or a family member gets sick. With the right knowledge and skills you can help members of your community to understand a disease, help them identify ways to prevent a disease from infecting them or their families, and make sure they know where to get care if they do get sick. As someone with special knowledge about how to protect your community, just by spreading the right information, to the right people, at the right time you can save lives and prevent ill-health.

This eCBHFA volunteer guide will equip you with the basics of Communicable Disease Prevention – how diseases are transmitted, and how to equip community members with the information and motivation to protect their families and communities from disease. No two communities, or even families are the same, and so the community understanding and ability to adapt our work is critical in this effort.

When communities are engaged and trained in epidemic preparedness and response, they become vital contributors to identifying and stopping outbreaks, saving lives, restoring services, reducing negative impacts, speeding recovery and building resilience.

Using the guides and toolkit

Communicable disease prevention volunteer guide

This guide is designed for community volunteers who work directly with community members. The guide contains key information and discussion points to work with community members in understanding risks involved across different disease transmission channels and how to minimise those risks.

The volunteer guide is comprised of seven topics which include knowledge, tools, talking points and activities for teaching communities in an interactive way.

Communicable disease prevention toolkit

The toolkit is designed to support both the Volunteer Guide and the Facilitator Guide with interactive tools, games and illustrative material. The toolkit is designed to be adapted and modified to local contexts, taking advantage of both local expertise and knowledge and differences in Ministry of Health approaches. Materials has been specifically designed to be printed in black and white or full colour, and with low-resource environments in mind.

Communicable disease prevention facilitator guide

This guide is designed for staff or trainers of trainers to conduct a training of trainers (ToT) or preparation course for staff and volunteers who will work directly with community members.

The Facilitator Guide allows volunteers hands-on experience with all the toolkit materials. Diagnostic tools to better understand community perceptions, barriers and individual concerns are also included.

Communicable disease prevention topics

Topic	Contents
1	Basics of communicable diseases
2	Transmission routes
3	Teaching disease prevention
4	Alerting authorities to possible disease outbreak
5	Basic health messaging

Topic 1

Basics of communicable diseases

Overview

Learn about what causes communicable diseases and how they spread. Understanding the reasons behind communicable disease transmission can help you prevent disease and help keep families and the community safe and healthy. Communicable diseases are caused by germs known as bacteria and viruses. There are multiple ways that communicable diseases spread. Myths about the spread of communicable disease can be dangerous. Knowledge about the multiple ways communicable diseases are transmitted can help prevent the spread of diseases.

Learning objectives

By the end of this topic, learners will be better able to:

- Define the volunteer's role in protecting their community from communicable disease spread
- Define vocabulary: communicable disease, germ, bacterium/a, virus, transmission
- Identify the causes of communicable diseases
- Understand the transmission routes of communicable diseases and the main diseases transmitted through these routes
- Define how myths about the spread of communicable diseases affect community health

Tools

- ❑ Basics of a communicable disease poster
- ❑ Transmission route posters (7)

Role of the volunteer

Volunteers learn in their eCBHFA orientation that they are a critical link between their community and the national health systems. The role of volunteers is to provide health and disease prevention information to their community whilst also informing the branch health coordinator or field officer when certain diseases or symptoms exist in a community or when many community members fall sick. Additionally, volunteers should inform the closest health facility. National Red Cross Red Crescent Societies should connect with national health systems at all levels while volunteers should establish a strong relationship with the local health facilities and health workers.

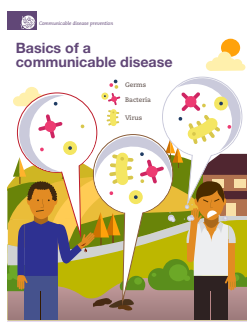
Volunteers are first responders when people are hurt or become sick. Trained eCBHFA volunteers can use the communicable disease prevention tools to recognize and respond to communicable diseases outbreaks that may occur in their community. Engaging with communities, listening to their perceptions and feedback, and involving them in decision-making is the best way to prevent and contain diseases before they spread to others.

Detecting an outbreak of disease early and acting quickly with the right response to control the disease is key to lessen the size, scale and impact of an epidemic. Diseases can quickly spread, affecting many people and causing sickness or death as well as fear and panic. Acting early with an evidence-based and community-focused collaborative approach to prevent and control the spread of disease is important.

Reflection questions:

1. How would you describe your role as a volunteer when thinking about communicable disease prevention in your community?

Causes of communicable diseases



□ Basics of a communicable disease poster

Communicable diseases are caused by germs. Germs are very small living creatures that we cannot see with our eyes. We would need a microscope to see germs. Communicable diseases are also called infectious diseases.

Communicable diseases are different from noncommunicable diseases like cancer or diabetes. Noncommunicable diseases are often caused by lifestyle choices and cannot be “caught” from another sick person.

Infections are caused by different types of germs which are called bacteria and viruses.

Bacteria are everywhere. Most bacteria are harmless. For example, bacteria live in our intestines and help us digest food. However, some bacteria cause infections in our throats, our lungs, our heart or other organs or in the bloodstream.

Viruses are smaller than bacteria and cannot live without a living host. A

living host means that viruses must live in a person, an animal or an insect. Most viruses cause diseases. Some people can be “healthy carriers” meaning they do not have any symptoms and are well but have the ability to transmit a virus such as HIV or cholera.

Germs are everywhere. Germs are in the air, in the ground and in the water, on plants and in and on animals.

Reflection questions:

- What are different types of germs called?
- What kind of diseases can germs cause?
- Where do you find germs?

Transmission routes

Transmission route posters (7)

Since germs are everywhere there are many ways to catch a disease. Both bacterial and viral infections are spread between people in similar ways. There are six ways that germs can spread:

Air droplet transmission occurs when someone coughs, sneezes or blows their nose so that germ droplets enter into the eyes, nose or mouth of another person. Main diseases that are transmitted through air droplet transmission include tuberculosis, measles, whooping cough, common cold, influenza, strep throat and meningitis.

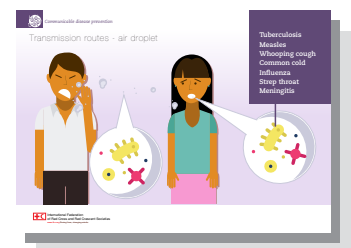
Faecal-oral transmission occurs when a person eats food or drinks water that is contaminated with faeces (poo) of infected people. The main diseases caused by contaminated food or water include cholera, shigella, rotavirus, salmonella, hepatitis A and hepatitis E.

Sexual transmission through unprotected sexual contact with an infected person. This can be through vaginal, anal or oral sex. Sexually transmitted diseases include HIV, syphilis, chlamydia, gonorrhea, herpes, hepatitis B, polio, Ebola and Zika.

Vector-borne transmission when infected animals or insects bite humans. Animals or insects that carry germs are called vectors. Vectors include mosquitoes, flies, ticks, rats, bats, monkeys and chickens. The main diseases caused by vectors are malaria, dengue fever, Chikungunya, Rift Valley fever and Zika. These diseases are caused by mosquito bites. Pregnant or women of reproductive age are most at risk for contracting Zika. Plague is spread by flea bites which are infected with germs from rats.

Blood-borne transmission occurs when a person comes in contact with infected blood. HIV, Ebola, hepatitis B and hepatitis C can be transmitted through contact with unsafe blood during a blood transfusion. Blood transmission can also occur through needlestick injury (sharing needles) or stab wound from dirty instruments such as scissors.

Mother to child transmission occurs during pregnancy or childbirth or through breastfeeding when an infection spreads from the mother to her baby. Zika can be transmitted during pregnancy while HIV, hepatitis B, syphilis, gonorrhoea or herpes can be transmitted during birth. Breast milk can transmit the HIV virus. Ebola can also be passed from mother to child



during pregnancy, and through breastmilk.

Although these are the transmission routes, we must remember that hands carry germs all of the time and are a common source of transmission of germs from one person to another.

More than one way!

Some diseases can be transmitted in more than one way. For example:

- Ebola, Marburg fever and Lassa fever can be transmitted through bodily fluids such as blood, vomit, saliva or mucous, urine, faeces, vaginal fluids or semen. This means that transmission can be through touching, air droplets, or the exchange of bodily fluids.
- Respiratory illness such as influenza or common cold can be transmitted by air droplets or touching someone who has the germs on their hands and then touching their own mouth or rubbing their eyes.
- Hepatitis C can be transmitted through sexual contact or a contaminated syringe.

As new and emerging diseases develop or cause outbreaks, we have to consider that new ways of transmission may be possible. Scientists continue to learn new facts about emerging diseases and what causes their outbreaks.

Dead bodies

Dead bodies can be highly infectious so care should be taken when handling them. Dead bodies from natural disasters do not cause epidemics because those deaths are usually a result of the trauma incurred. However, when deaths have occurred as a result of a highly infectious disease such as Ebola, Lassa fever or cholera, great care should be taken because of the possibility of spreading the disease. Bodily fluids or blood splashed into eyes, mouth or nose that makes direct contact into an open cut of someone handling the body poses a very great risk of them contracting the disease.

Reflection questions:

1. What does transmission route mean?
2. How many different ways can disease be transmitted?
3. What does faecal oral transmission mean?

Myths about communicable diseases

Many myths and rumours exist about communicable diseases even though scientists know that germs cause the diseases and know how germs are transmitted. Myths and misconceptions about communicable diseases can be dangerous.

Myths and rumours often occur when people do not have enough information and understanding about a disease, or they can be linked to strong cultural beliefs. Incorrect beliefs that disease is caused by witchcraft or curses may cause some people to avoid going to the health centre when they are feeling unwell. Some people may fear being told that they have a disease so they ignore how they are feeling. Myths and rumours can be very challenging to address and change. This means that understanding disease transmission becomes vital in order to change the myth or misconception. If an infected person does not go to the health centre or ask for help they may spread the disease to other people.

There are many common myths and rumours about communicable diseases. Here are a few examples:

- Some people believe that eating mangoes can give you malaria.
- Some people think that HIV can be transmitted through contact such as holding hands with an infected person. This is not accurate. HIV is only transmitted through sexual contact or direct blood contact.
- Some people believe in conspiracy theories or that western medicine makes sickness worse or kills people.
- Some people believe TB comes from a neighbour casting a spell and recommend a trip to the sorcerer to remove the spell.

Reflection questions:

1. What myths have you heard in your community about infectious diseases?
2. How can myths hurt the health of your family or community?
3. What are the risks if myths are not addressed or discussed?

Topic 2

Transmission routes

Overview

Learn about communicable diseases by their transmission routes. An introduction to tools to support teaching community members about communicable disease prevention and control.

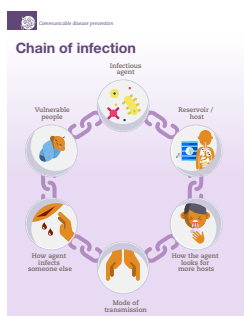
Learning objectives

By the end of this topic, learners will be able to:

- Identify key diseases for each transmission route
- Explain how to keep from becoming sick from a communicable disease based on how the disease is spread
- Explain the chain of infection and how to break it
- Identify specific communicable diseases that may affect your community, appropriate prevention and control measures, and the signs and symptoms of specific diseases
- Define the volunteer's role in protecting their community

Tools

- ❑ Chain of infection poster
- ❑ Disease tools (27)

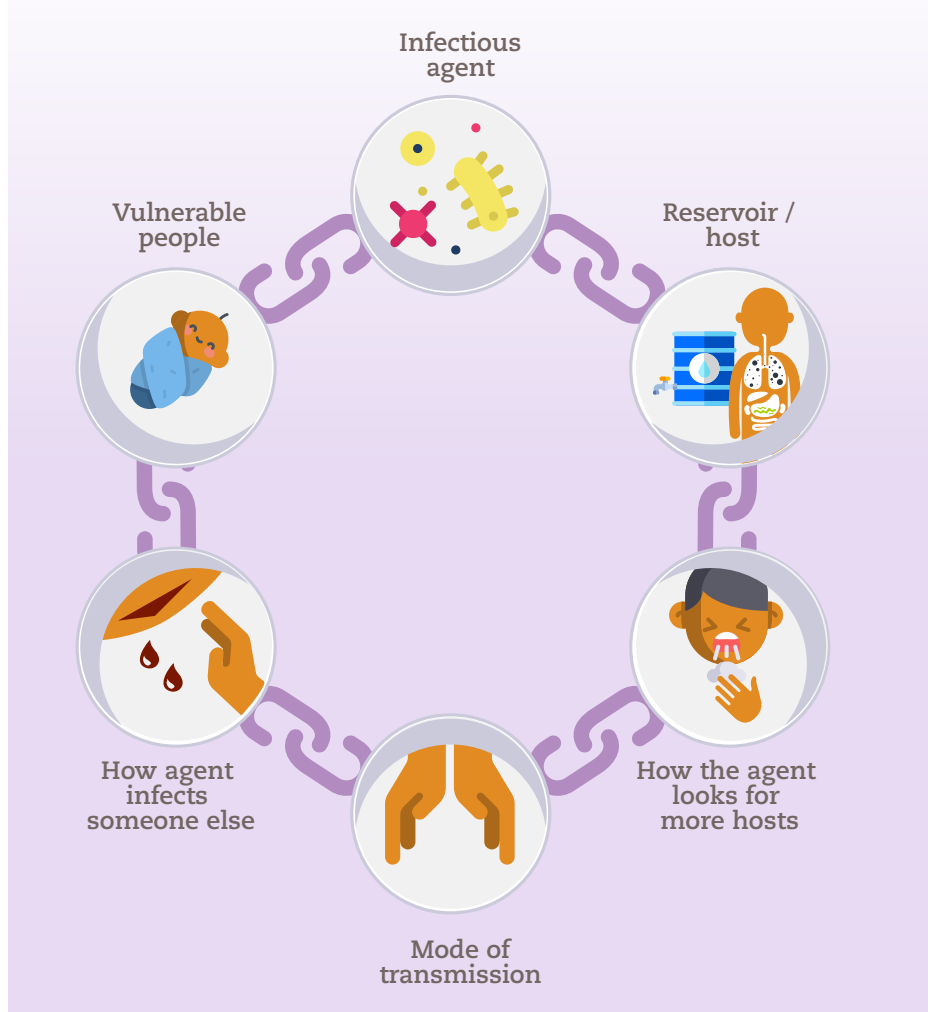


Transmission of communicable diseases

- ❑ Chain of infection poster

The transmission of infectious disease is described by a chain of six elements also known as **the chain of infection**. All six elements are present when infection or disease shows up in an individual. By eliminating just one element within this chain of infection, we can interrupt the transmission of a communicable disease.

Chain of infection



The six elements on the chain of infection include:

1. Infectious agent – What causes the infection?
2. Host / where infectious agent lives – Where does the infection live and reproduce?
3. How agent looks for more hosts – How does the infection stay in the body while looking for more hosts?
4. Mode of transmission – How does the infection spread to others?
5. How agent infects someone else – How does the infection enter a new host?
6. Vulnerable host – Who will get sick from the infection?

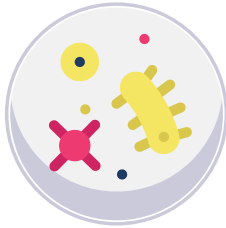
For example, on the yellow fever disease tool, we know that “vulnerable hosts” can be protected with vaccination, and vector control methods can be used to reduce the likelihood of transmission and even block transmission (e.g. mosquito

nets and personal protection against mosquito bites). For other infectious germs, such as viruses or bacteria, these actions may not be as effective. It is helpful to study the disease tools and learn about symptoms, hosts and the elements in the chain of infection to break the cycle of transmission. First let us look at the different elements in the chain of infection.

Understanding the chain of Infection

1. Infectious agent - What causes the infection?

Infectious agents are bacteria, viruses, fungi and parasites. It takes just a small amount of the agent to cause an infection. For example, a few bacteria can cause bloody diarrhoea (shigella) while other diseases may require hundreds of thousands or millions of bacteria. It all varies by disease.



2. Reservoir / Host – Where does the infection “hide” or “live”?

The reservoir / host is the place where an infectious agent or germs live and grow. This can be inside a living person or an animal, or in a substance like water. For example, humans are the reservoir for measles. The measles virus passes from an infected person directly to another person. Another example is Lassa fever where rodents are the reservoir. The Lassa virus is contracted via contact with food contaminated with rodent urine or faeces (poo).



The host may be a person or an animal where the germs are living and the reservoir shows signs of sickness. Other times a person or an animal may look completely healthy but is carrying the germs. For example, 80 per cent of people infected with cholera do not develop symptoms. While a person or an animal may not feel or look sick, they can transmit the germ, bacteria or virus to others. These people or animals are called carriers and they often make others sick without even realizing it. The people who receive the infection are called hosts.

3. How the agent looks for more hosts - How does the infection leave the host?

The way that the infectious agent or germs leave the host to go on and spread to other people are through one or more of three ways:

1. the respiratory tract through coughing, sneezing or kissing.
2. body fluids and waste including urine, sexual fluids, faeces, breastmilk and blood.
3. through the skin.



4. Mode of Transmission – How does the infection spread to others?

There are many ways in which disease can spread. Understanding the source of infection and how it is transmitted is key in being able to prevent and control epidemics. There are many routes of transmission that allow diseases to spread. These routes can be described as direct transmission or indirect transmission.



Direct transmission involves direct contact through skin-to-skin contact, kissing, sexual intercourse and blood. Direct transmission can also occur through saliva droplets from sneezing, coughing and talking. The source of the infectious agent may be a person or an animal.

Indirect transmission occurs in ways other than by actually touching the infected host. Infectious agents may be spread in the air where germs are in dust or in the air itself. They can be spread through touching contaminated objects such as food, water or utensils. Indirection transmission can also be vector-borne via insects or animals.

5. How the agent infects someone else – How does the infection enter a new host?

The infectious agent enters a new host in different ways. Transmission can occur through the respiratory tract from inhaling germs or kissing; through the mouth by consuming infected food or water; through the skin via insect bites or cuts; through the mucous membrane via contact of eye and mouth with dirty hands; through the blood via a blood transfusion or needle-sharing; and from the mother's placenta across to the baby.



6. Vulnerable host – Who will get sick from the infection?

A vulnerable host is a person at risk of being infected. The level of vulnerability depends on a person's age, gender, ethnicity and genetics, immunity and other factors that affect a person's ability to resist infection. For example, a person who is weak with undernutrition has less immunity to resist infections. Alternatively, a child who is immunized is protected and not vulnerable to certain diseases. A child who is not immunized may be vulnerable to communicable diseases in an emergency, e.g. measles or cholera.



Reflection questions:

1. What are six elements of the chain of infection?
2. Explain one method of direct transmission of infectious agents.
3. Explain one method of indirect transmission of infectious agents.

Preventing communicable disease by breaking the chain of infection

Communicable diseases can be prevented by breaking the chain of infection. In other words, breaking one of the links of the chain will prevent the spread of the disease. This is why it is so important to practise healthy behaviours and block the transmission route in appropriate ways.

To block some airborne transmissions, we ask people to cough into their elbows or practise social distancing to avoid standing, sitting, or talking too close to others. Depending on the disease the appropriate social distance may be one to two metres away.

Sexual disease transmission can be blocked through the use of a condom during intercourse or sex.

Some faecal-oral and water-borne diseases can be prevented when we routinely wash our hands with soap and water and do not touch our hands to our mouth, nose or eyes before washing. Additionally, prevention occurs when we practise good food hygiene and safe sanitation, boil unsafe water before drinking or using water in the preparation for cooking.

To block some blood transmissions, we advise people to wear gloves when touching another person's blood.

Some vector-borne transmissions can be prevented by the regular use of mosquito netting or screens which prevent mosquito bites.

To block some mother-to-child transmissions, we advise HIV+ mothers to regularly take their medicine before, during and after birth..

Reflection questions:

1. What are the best ways to prevent the spread of vaccine-preventable diseases?
2. What are the best ways to prevent the spread of airborne diseases?
3. What are the best ways to prevent the spread of faecal-oral diseases?
4. What are the best ways to prevent the spread of sexually transmitted diseases?
5. What are the best ways to prevent the spread of vector-borne diseases?
6. What are the best ways to prevent the spread of bloodborne diseases?
7. Who are vulnerable people or groups in your community?
8. How can eCBHFA volunteers identify and ensure vulnerable groups in your community stay safe from communicable diseases?

Exploring specific infectious diseases



□ Disease tools (27)

The disease tools are brief guides that give an overview of common communicable diseases that many communities may experience. While not all tools will apply to your community, the tools allow volunteers and their community members to become familiar with some of the most common diseases, symptoms to be aware of, and prevention and control measures that can be used to protect people from sickness. Each disease tool also shows a brief description of the disease, the symptoms of the specific disease and prevention and control methods.

Work with a partner, a team or individually to explore the disease tools using the following set of questions to better explore the content.

1. What are the key diseases for each transmission route?
2. Which diseases have you heard of before?
3. How can you refresh your knowledge of the symptoms, treatments and prevention and control methods for each disease?
4. Which animals are unsafe to keep around your home or are unsafe to eat? What diseases can be caused by animals? What can you do to prevent people from getting sick from animals around your home? What can you do to prevent sickness if you have or eat animals?

At the bottom of each tool are questions or prompts to help eCBHFA volunteers ensure that Red cross Red Crescent is identifying and meeting the needs of the most vulnerable or excluded groups in their communities. Discuss how you can use this information to make stronger and more resilient communities

Disease tools reflection questions:

1. Which diseases should you focus on for your community? Why have you chosen these?
2. Which diseases are not applicable to your community? Why?
3. How might you use the tools with your community to teach about the disease?

Topic 3

Teaching disease prevention

Overview

Use assessment tools to identify when volunteers should teach communicable disease prevention based on seasonal calendar and other assessments. Communicable diseases that occur regularly in a community can be prevented through teaching about disease prevention before an outbreak occurs. Volunteers must first identify communicable diseases endemic to their community. Through the use of a community's seasonal calendar volunteers can identify how natural weather patterns and community activities can pose potential health risks for each season, activity and festival. Using this information can then help volunteers determine when to teach about disease prevention and control.

Learning objectives

By the end of this topic, learners will be better able to:

- Identify which communicable diseases are endemic in your area
- Identify when to teach communicable disease prevention based on an seasonal calendar
- Identify which groups within the community will be trained in communicable disease prevention

Tools

from Assessment module

- ❑ Seasonal calendar for your community
- ❑ Focus group discussion outputs
- ❑ Community action plan
- ❑ Community mapping

Identifying communicable diseases in your community

When a disease is commonly seen in an area it is called endemic or familiar to that particular area or country. Often the term epidemic or the sudden outbreak of a communicable disease is confused with endemic. Endemics are communicable diseases that are constantly present in a community while epidemics are sudden outbreaks of the disease. A pandemic is the worldwide spread of a new disease. While eCBHFA volunteers are critical to keep communities healthy and informed if and when a pandemic does occur, the majority of eCBHFA volunteers' communicable disease prevention work is to keep community members safe from endemic or regularly transmitted diseases.

It is possible for a community to eradicate or rid itself of a communicable disease that is endemic to the area. Ridding a community or country of an endemic communicable disease requires focused work. A community or country would have to work at the root causes of the communicable disease, identify its transmission routes, and work constantly to break the chain of infections through public health activities. For example, in 2015, after years of effort, Nigeria announced that polio was no longer endemic in the country. In the event of a disaster, prevention and disease awareness activities implemented early can help decrease the risk of disease outbreak. For example, if a case of measles is detected following a border crossing it is vital that the Ministry of Health raises the alert so social mobilization activities can start to raise awareness amongst communities.

To identify the diseases that are endemic in your area you can work with the Ministry of Health or local health system. If your community borders another country, learning about the diseases in your sister communities across the border is also helpful to help contain any communicable disease. This was the case when in 2015 the porous borders in Guinea, Liberia, and Sierra Leone saw the movement of sick people leading to a spread of Ebola. By working with the local health authority and National Societies volunteers can learn what disease tools are the most important to teach to their community members.

Reflection questions:

1. What does endemic mean? What does epidemic mean?
2. What communicable diseases are endemic in your area? How do you know this?
3. How can you determine what other communicable diseases are considered endemic in your area?

Using your community's seasonal calendar to determine when to teach communicable disease prevention

It is important to know where and when communicable diseases have affected your community in the past and what efforts were helpful in stopping transmission. You can refer back to the Assessment tools especially the seasonal calendar from the Assessment Module to understand disease patterns and all aspects of past disease outbreaks.

From these assessment tools you can learn:

- Which communicable diseases occur in your community
- Who in your community has been most affected
- Where specifically communicable diseases occur
- When communicable disease outbreaks occur

It is important to note all aspects of the cycle of infection and identify the communicable diseases, people, and timing when considering disease prevention efforts. By reviewing or developing a seasonal calendar with the community and local health authorities, you can learn when people in your community are at the most risk of becoming sick. A seasonal calendar is a guide the volunteer can use to prepare for each season and potential risk factors. Seasonal calendars should always be completed at the community level.

Your community's seasonal calendar can inform you about natural weather patterns that can directly affect community health, such as blizzards, monsoons, heat waves, droughts, cyclones, and hurricanes. The calendar should also include religious holidays, festivals and other scheduled activities that can affect your community's health. By considering what diseases are most likely to occur at certain times of the year, volunteers can determine when to teach prevention or send health messages to their community.

For example, through a review of a community's seasonal calendar a volunteer could identify:

- The rainy season so volunteers and communities can take extra actions to control mosquitos and prevent certain vector-borne diseases such as malaria
- Certain weather conditions that may affect food shortages causing under-nutrition thus making community members more susceptible to infections
- When community members may be sharing food at festivals, weddings, or other celebrations where a lack of clean water is possible increasing the possibility of a diarrhoea outbreak

Take a look at the sample seasonal chart below and consider the following example: Simon is an eCBHFA volunteer in his community. For the last five years, after every monsoon season, there is an outbreak of watery diarrhoea. When should Simon start teaching about diarrhoea prevention?

Topic	Jan	Feb	Mar	Apr	May	Jun	Jul	Aug	Sep	Oct	Nov	Dec
Floods												
Monsoon/rainy season												
Ramadan / Eid												
Christian Orthodox Festival												
New Year												
Harvest												

If you guessed January, you are correct. By preparing his community before the threat of disease even occurs with the monsoons, Simon is preparing and preventing transmission of infectious agents that can contribute to a watery diarrhoea outbreak. His community is better prepared and buys soap, is reminded to practice safe household water treatment and sanitation and remembers the critical times for handwashing as well as the misery of having diarrhoea in a household so household members can stop transmission before it occurs.

Looking at the seasonal chart answer the following questions based on what you know about these types of events and the potential for communicable disease outbreaks:

- In what ways could a community be affected by these events? What communicable diseases may occur?
- When should a volunteer teach prevention methods or send a community health message?

Remember by understanding the risks and the diseases in your community and their relation to the seasons, a volunteer can better prepare for and try to prevent and control sickness. To do this, volunteers and communities should develop and regularly refer to the seasonal calendar.

Reflection questions:

1. What should be included in your community's seasonal calendar?
2. How can you use your community's seasonal calendar to identify when to teach about communicable disease prevention to your community?
3. How can you use the events in the calendar to anticipate behaviours that may increase the risk of spreading a communicable disease?

Identifying groups within the community to be trained in prevention

Once you have determined when to teach communicable disease prevention, volunteers need to identify who specifically they need to train or include in their health messaging.

In our example above, Simon knows that watery diarrhoea occurs after the monsoon season. Now he needs to determine who to train about its prevention. Simon talks to the nurse at the local clinic and learns that children under five years of age were the ones most likely to suffer and die from watery diarrhoea during every monsoon season for the last five years. Simon knows that mothers are the best people to receive messages about keeping their young children safe from disease and that the families by the riverbed are the most affected. Simon decides that mothers of young children especially those living near the riverbed are the primary group he should speak with about preventing diarrhoea after the monsoon season.

Again, looking at the sample seasonal chart above answer the following questions based on what you know about these types of events and the potential for communicable disease outbreaks:

- Who in the community may be affected by the events included in the calendar?
- What are the different ways a volunteer can you determine who is most affected by these events and should be targeted for disease prevention?

Reflection questions

1. How can eCBHFA volunteers identify the target groups to be trained in preventing communicable diseases?
2. What are some activities that Simon can do before monsoon season as well as during the monsoons to keep people healthy?
3. What can Simon do or say if he sees community members not washing their hands properly or inappropriately storing or using unclean water?

Topic 4

Alerting authorities to possible disease outbreak

Overview

General guidance for volunteers on what symptoms, rumours or other occurrences should prompt a volunteer to alert a health authority of possible disease outbreak. Using the community mapping tool from the Assessment module can help volunteers identify disease risk areas, vulnerable people within your community and community resources. The tool can help you be prepared in case of a disease outbreak or other health emergency. As a volunteer, one of your roles is to alert the authorities if you hear of an odd rumour or learn of an unexpected death or illness. The ability of a volunteer to report to the branch or local clinic can significantly help limit the disease outbreak and help in the prevention of diseases spreading.

Learning objectives

By the end of this session, learners will be better able to:

- Develop or modify a community map that includes disease risk areas, vulnerable groups and community resources
- Identify when to report odd rumours or unexpected illnesses or deaths, and to whom these should be reported

Tools

- ❑ Community mapping tool (Assessment module)
- ❑ Sample community map
- ❑ Alerting the authorities game

Mapping communicable disease risks, vulnerabilities, and resources

□ Sample community map

In the Assessment module, you used community mapping as an assessment tool to identify resources within your community. If the map your community created is recent (less than three months old), this is the one to use and review with your community. If the community does not have a community map that was developed with volunteers or if the map is more than three months old, refer to the Assessment module to develop a new, updated map for your community.

When reviewing or drawing the community map be sure to ask community members to discuss and answer probing questions like the examples below. Based on the responses, you can prompt community members to add important details to the community map. By asking questions, volunteers can work with the community to map disease risks, determine vulnerable groups and locate resources to better prevent communicable disease.

In the toolkit, there is a sample community map and three icons you are asked to place on the map to show areas that are:

- Risks
- Vulnerabilities
- Resources

Using your actual community map or by referencing the sample community map, discuss where the risks, the vulnerabilities and the resources are in this community. Be able to explain why you think so.

Prompt questions may include:

- Where are the local health facilities and services? Include traditional community healers.
- Are there skilled health workers in your community?
- Can people access health care at weekends or at night?
- What are the habits, practices and beliefs regarding hygiene, sanitation and water in the community?
- Who are the vulnerable people in the community?
 - How many children in the community are aged five or fewer years?
 - Are there pregnant women in the community?
 - Are there disabled people in the community?
 - Are there older people in the community?
 - Are there people without identification documents in the community?
 - Are there refugees or displaced people in the community?
 - Are there nomadic or transient groups near the community?
- Where do vulnerable people in your community live or work? How can volunteers access vulnerable people and how often?

- Where do people learn new information about illness and treatment? Where do people go for treatment?
- Where do people learn about prevention measures?
- When someone in the community gets very sick and they die, what happens? Who is alerted? How can volunteers ensure Ministry of Health authorities are aware of any unusual illnesses or patterns of sickness?
- Are children in the community vaccinated?
- Do new mothers typically breastfeed their babies?
- Where are the pregnant and lactating women?
- Where are the mosquito breeding sites in the community?
- Does the school have water and sanitation facilities including soap and handwashing stations?
- How many people live in a household? Do homes have proper ventilation? Do people cook inside the house?
- In an urban setting, how do people interact or share information?

Reflection questions:

1. What is the purpose of developing a community map?
2. How can you determine who is vulnerable in your community?
3. What other prompt questions could you ask when completing your community map?
4. What steps will you take to develop or modify a map in your community?

Alerting the authorities

As a volunteer, one of your roles is to alert your branch supervisor or local clinic if you hear of an odd rumour or learn of an unexpected death or illness. For example, if you hear that an adult man died from diarrhoea, a volunteer may understand that generally, adults do not die of diarrhoea. You may consider this an unexpected death, so in this example, you should consider reporting this death to your branch supervisor or local clinic. This reporting will ensure that someone will provide support or even investigate if something seems 'not right'. Many times, new infectious diseases are found this way and by keeping alert for any unexpected sickness or deaths, eCBHFA volunteers can help to prevent the spread of a new or unexpected disease. The health authorities will determine if the death or illness is explainable.

An eCBHFA volunteer may go to the person's house to provide psychosocial support and see how the family can keep themselves free from sickness. During the household visit, the volunteer may take note of how household water is stored and treated, if there is soap in the house, if there are vectors or pests in the house, if other people in the family are also feeling ill. Volunteers can discuss household water treatment options or how communicable diseases spread or help to find care for others who are sick.

Health authorities

If your health authorities determine there is a health emergency the government is in charge of declaring a “state of emergency” and whether they want assistance from within the country, regionally, or require broader international assistance. Your Ministry of Health will take the lead during the health emergency.

The World Health Organization (WHO) provides close support to the Ministry of Health to either help strengthen the response or assist in areas of capacity-building such as surveillance, case management, data analysis, case definitions and reporting. All other agencies including Red Cross Red Crescent report to the Ministry of Health to ensure that their response is in line with the country response. At times the government may ask the Red Cross Red Crescent to play a specific role. For example, the government may ask the National Society to be involved with a field hospital, mobile clinics or community social mobilization activities.

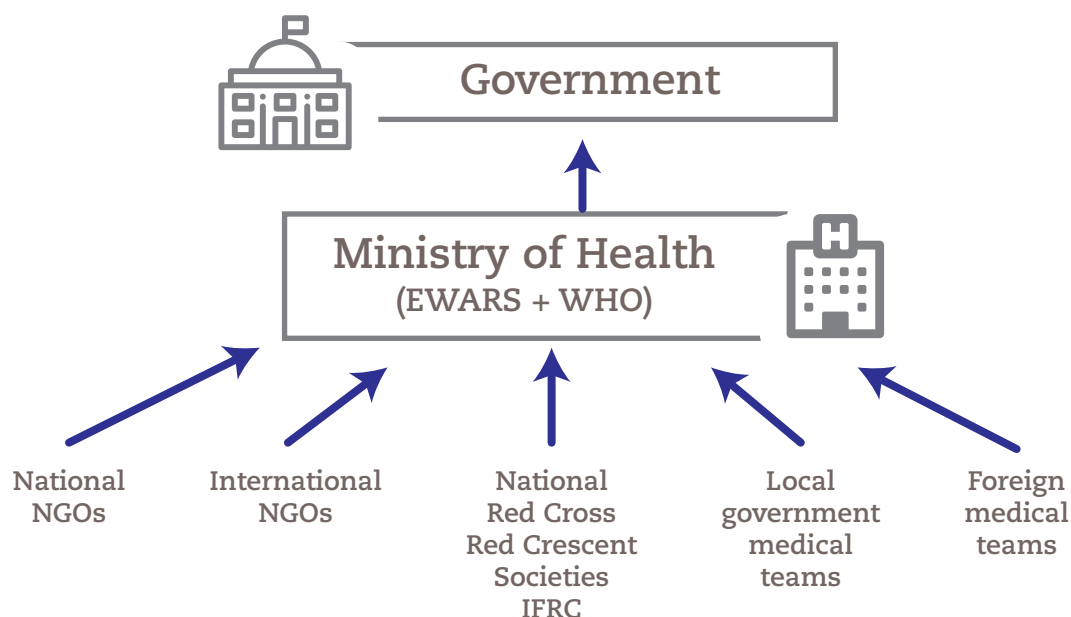
Surveillance

During an emergency the public health surveillance system will be enabled. The surveillance system is called the [Early Warning and Reporting System \(EWARS\)](#). The primary focus of EWARS is to strengthen the early warning alert and response during emergencies. This system supports the MoH/DoH with WHO providing assistance to strengthen capacity to handle the emergency.

Additionally, EWARS play the following important roles:

- Surveillance of cases and deaths
- Collection and analysis of information provided
- Regular reporting to the government

Surveillance is the collection of relevant information for analysis and action. For example, in a cholera outbreak it is important to know where the cases are located and determine if the outbreak is spreading into new areas or growing in numbers. Surveillance also includes noting the interventions in place and



documenting if they are effective. For example, are there social mobilization activities about disease awareness and hygiene promotion? Again, in the case of a cholera outbreak, are there cholera treatment centres? This type of information helps the Ministry of Health prioritize and plan the appropriate response to the outbreak. Evaluating the information helps assess whether the response has been effective. It is important that all agencies feed their information (numbers) to EWARS. Evaluating the information helps assess whether the response has been effective. It is important that all agencies feed their information (numbers) to EWARS.

Reflection questions:

1. When should volunteers alert authorities to possible disease outbreak?
2. What organization in your community or country takes the lead in health emergencies?

Let's play!

Alerting the authorities game

Now we can play a game that will help you understand your role in linking community issues with health authorities. This game will show how volunteers and informed community members can help to save lives by identifying when an issue may need to be addressed or reviewed for potential public health concerns.

After the game play, discuss how eCBHFA volunteers form the important link between community and health authorities.

Topic 5

Basic health messaging

Overview

Basic health messaging guidance includes simplicity, a call to action and communication of a benefit. eCBHFA has many materials that can be used to teach about health and community. But each community is unique and has its own problems, behaviours, norms and reasons that might contribute to unhealthy behaviours or sickness. Volunteers should tailor messages to their community; there is a general set of guiding principles to use when developing written or verbal health messaging. The guidance is simple and should be applied when delivering health content or developing or revising health materials or campaign materials.

Learning objectives

By the end of this topic, learners will be better able to:

- Explain the use of the doer/non-doer analysis tool for understanding behaviours in the long term and the short term
- Understand the components of a basic health message
- Evaluate basic health messages using the effective message scorecard
- Design a basic health message for the prevention of a communicable disease for a specific community

Tools

- ❑ Doer/non-doer analysis tool for behaviour change (from the Assessment module). The assessment module can be found at ifrc-ecbhfa.org/guides-and-tools/
- ❑ Effective message scorecard
- ❑ Sample messages - two messages as found in the toolkit and one downloadable cholera video. Download the video in your language from <https://global-healthmedia.org/animations/>

Identifying problem behaviours

In the eCBHFA Assessment module, one of the newest assessment tools is the doer/non-doer analysis tool. The tool helps volunteer teams learn why people in a given community (doers) are already doing the target behaviour while those around them (non-doers) – in the same environment – do not practise the healthy behaviour. This tool looks at the reasons why problem behaviours exist.

This tool can be used to gather evidence for what makes the doers in a community practise a specific healthy behaviour and use that information to teach the non-doers how they can improve.

The tool allows volunteer teams to assess for long-term or development behaviours like healthy eating and how to target interventions to get everyone to do the target behaviour. The tool can also assess for short-term or emergency behaviours like handwashing when many people are becoming sick with watery diarrhoea, and identify ways to target interventions to get everyone washing their hands appropriately and at the right times.

By identifying the causes or barriers associated with the behaviour, we start to build an evidence base on which we can develop tailored and appropriate health messaging and tools that better target and change problem behaviours.

Reflection questions:

1. How have you used the doer/non-doer analysis tools in the past?
2. Considering what you have learned about communicable disease prevention, what behaviours might you be analysing in your community in the future?

Health messages

❏ Effective message scorecard and Sample messages

Any health message should be developed or revised based on this information (evidence) collected about community knowledge and practices. While you can use already existing health messages from Red Cross Red Crescent and from the Ministry of Health, these, **MUST** be adapted for your target audience. Your evidence base is your first step in this regard.

There are **four criteria that effective health messaging** should have:

1. Simple and understandable wording
2. A stated benefit (related to what motivates target audience to change behaviour)
3. A call to action (what behaviour do you want them to practise?)
4. Informed by local culture, community beliefs and local practices, and tested within the community

Once drafted, messages must be **checked for quality and for suitability**. For example, do a review by giving the messages to selected community members. Discuss how different community members are likely to interpret the messages.

Ask questions such as:

- Are there possible misunderstandings? Are the language and terms suitable?
- What meaning did they take away from the messages? Does this match your intended meaning?
- Is the message sensitive to the culture? Will it cause offence?

In an emergency it is always important to check the key messages that the Ministry of Health wants delivered..

Reflection questions:

1. What are the four criteria that effective health messaging should have?
2. Why is it necessary to check a draft message for quality and suitability?

The fundamental principles of the international Red Cross and Red Crescent movement

Humanity The international Red Cross and Red Crescent movement, born of a desire to bring assistance without discrimination to the wounded on the battlefield, endeavours, in its international and national capacity, to prevent and alleviate human suffering wherever it may be found. Its purpose is to protect life and health and to ensure respect for the human being. It promotes mutual understanding, friendship, cooperation and lasting peace amongst all peoples.

Impartiality It makes no discrimination as to nationality, race, religious beliefs, class or political opinions. It endeavours to relieve the suffering of individuals, being guided solely by their needs, and to give priority to the most urgent cases of distress.

Neutrality In order to enjoy the confidence of all, the Movement may not take sides in hostilities or engage at any time in controversies of a political, racial, religious or ideological nature.

Independence The movement is independent. The National Societies, while auxiliaries in the humanitarian services of their governments and subject to the laws of their respective countries, must always maintain their autonomy so that they may be able at all times to act in accordance with the principles of the Movement.

Voluntary service It is a voluntary relief movement not prompted in any manner by desire for gain.

Unity There can be only one Red Cross or Red Crescent Society in any one country. It must be open to all. It must carry on its humanitarian work throughout its territory.

Universality The international Red Cross and Red Crescent movement, in which all societies have equal status and share equal responsibilities and duties in helping each other, is worldwide.

For more information on this IFRC publication, please contact:

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